

COMPETITIVE ANALYSIS · CONSOLIDATED · V4

The bones & joints menopause brand Asia forgot.

How Bonefirm justifies an **SGD 8.50/day** premium as the fair, single replacement for the supplement stack — and the 90-day path to own the **bone/joint × menopause** intersection no incumbent sees.

32.9%

joint pain = #1
menopause symptom,
SG women

18.6%

hot flushes rank only
5th (18.6%)

SGD 8.50

Bonefirm price/day —
inside the stack range

0→

premium structural slice,
owned by nobody

ONE-SENTENCE CLAIM · BUYER LANGUAGE

"Your bones and joints change during menopause — and Asian women feel it as **joint pain first, not hot flashes**. Bonefirm looks after your bones and joints through menopause, **without hormones**."

DATE

2026-08-20

METHOD

Positioning Theory · TCO test · Category Trap

PREPARED BY

ObserveCo · Design System v.consulting

§ Executive Summary — the core thesis

Bonefirm is the product built around the symptoms Asian women actually experience — **joint pain first, silent bone loss beneath it**. It justifies its premium (SGD 8.50/day) as the fair, single, one-stop replacement for the stack women already buy separately.

THE WEDGE

NUH/NUS research: **joint pain #1 (32.9%)** for Asian women. The market sells the 5th-ranked symptom (hot flushes). **Bonefirm serves the #1.**

THE BEACHHEAD

The **silent-risk-aware woman** — triggered by a DEXA scan, osteopenia, or a parent's fracture; already paid SGD 130–150 for the scan. Nobody serves her.

THE MOAT

The **founder story + education engine** (book, RC talks, conference) — the one durable, hard-to-copy asset. The moat is trust, not the formula.

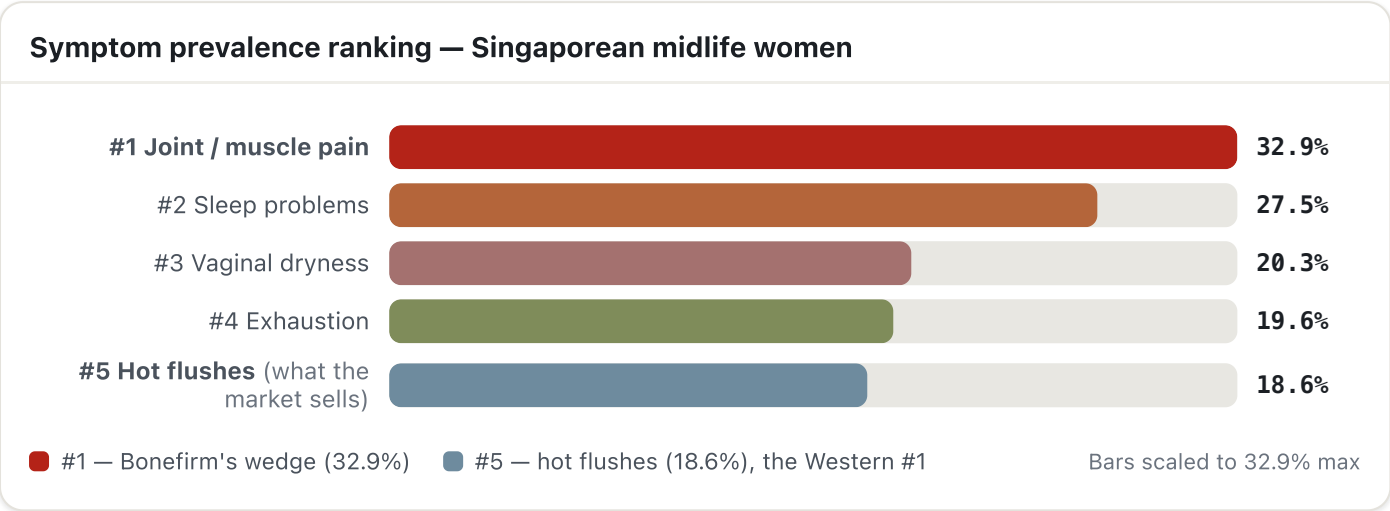
How SGD 8.50/day is justified — the premium, made fair & culturally anchored		
LEG	ARGUMENT	EVIDENCE
Leg 1 — Fair value Parity, not cheap	The stack Bonefirm replaces (bone + joint + collagen + mood) runs SGD 5.80–9.80/day . SGD 8.50 sits inside that range — the premium is not a mark-up.	SGD 5.80–9.80 stack (validated SG retail)
Leg 2 — One-stop-shop	One product, one cause, one accountable formula instead of 4–5 scattered products. The East–West integration is proprietary.	NEM + Danshen + Chuanxiong + Dendrobium
Premium anchor — TCM	Women who trust TCM already pay SGD 7–20/day for the same root cause. SGD 8.50 sits in that same premium band — culturally anchored.	SGD 7–20 TCM herbal / day
One open gate (B1): prices are validated; whether women actually buy the full stack (4–5 products) or only 1–2 is not. Verdict: feasible, with one condition (B1) .		

THE DESTINATION · POSITIONING

Bonefirm occupies the "**bones and joints through menopause, for Asian women**" position — the brand built for the Asian woman's body: the silent bone loss and the #1 joint pain, justified at premium by being the fair, culturally-anchored, single replacement for the stack.

1 The wedge in data — joint pain is #1, not hot flushes

NUH/NUS IWHP study (Maturitas 178:107853, 2023 · PMID 37806008), 1,054 midlife SG women. The market sells the 5th-ranked symptom; Bonefirm serves the #1 and the silent risk beneath it.



SECOND HEADLINE NUMBER

74.9%

of midlife SG women report some form of arthralgia
(Study 2, Climacteric 2023 · PMID 38099561)

THE OVERTURN

The medical community assumed hot flushes are #1.
NUH's Prof Yong Eu Leong: for Asian women, joint/muscle pain is #1 and hot flushes rank only 5th.
This is published, peer-reviewed, SG-specific.

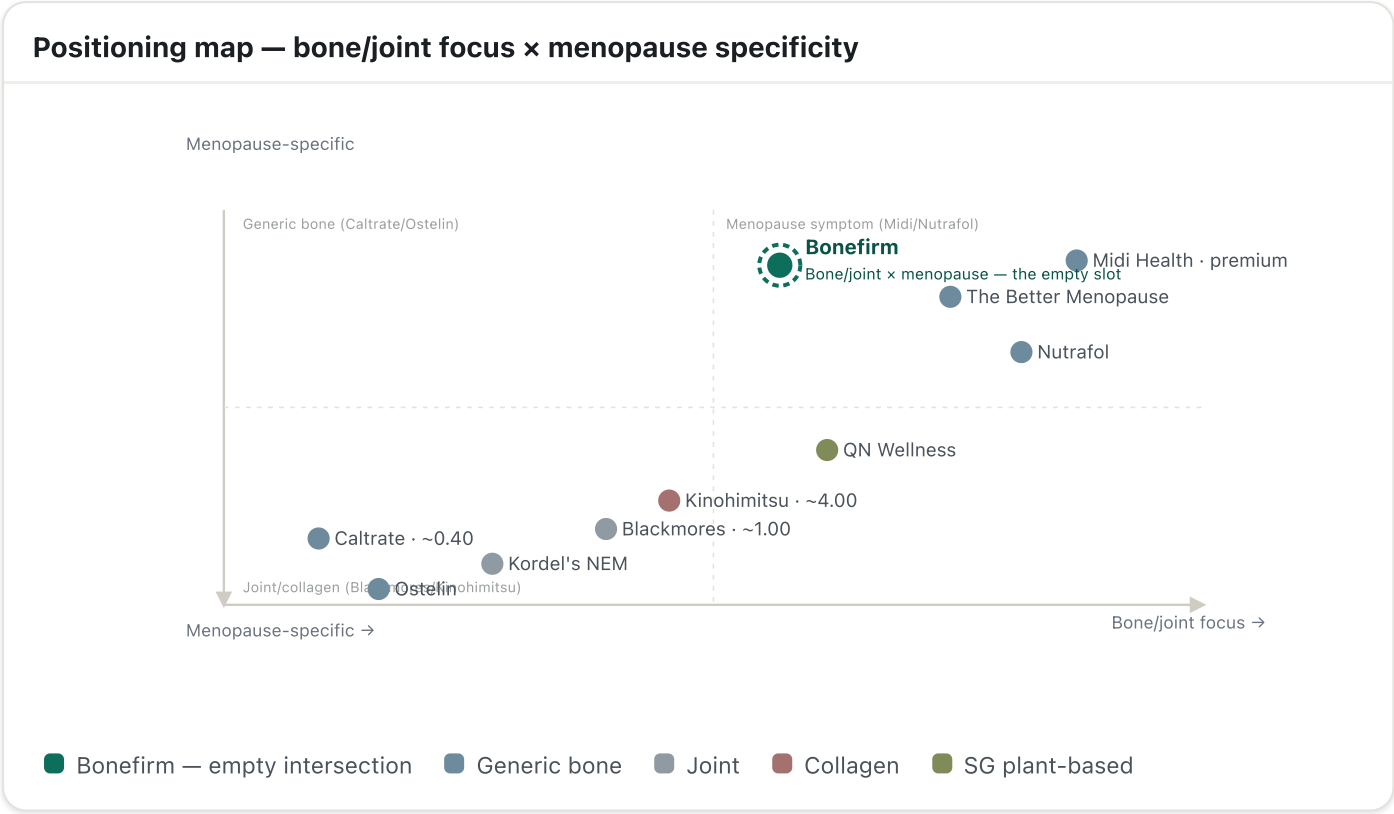
2 The correct customer — "Women 45+" is a demographic, not a customer

#	SEGMENT	TRIGGER	WILLINGNESS TO PAY	SERVED BY?	VERDICT
1	Silent-risk-aware woman BEACHHEAD	DEXA scan / osteopenia / parent's fracture	High — already paid for DEXA	Nobody	The beachhead
2	"I feel it" woman (32.9%)	Current joint/muscle discomfort	Medium — compares vs glucosamine	Blackmores, Kinohimitsu, Kordel's	Expansion — largest, contested
3	Daughter / caregiver	Parent's fracture or decline	High (emotional)	Nobody	Secondary buyer (founder's origin)

Beachhead rationale: the only segment that (a) understands the "silent risk" framing, (b) is driven by fear of fracture, (c) is genuinely underserved, (d) will pay premium.

3 Competitive set & the empty intersection

Bonefirm is **#5+ on the menopause ladder**, **#8+ on the bone/joint ladder**, **<1% of either** — fringe. But it is the **only brand at the bone/joint x menopause intersection**, empty because incumbents don't see it, not because it's unprofitable.



Competitive capability matrix							
CAPABILITY	BONEFIRM	CALTRATE	BLACKMORES	KORDEL'S	KINOHIMITSU	MIDI/NUTRAFOL	QN
Menopause-specific	✓	✗	✗	✗	✗	✓	✓
Bone/joint focus	✓	✓	✓	✓	✓	●	✓
Bone/joint x menopause intersection	✓	✗	✗	✗	✗	✗	✗
Non-hormonal	✓	✓	✓	✓	✓	●	●
NUH/NUS SG data	✓	✗	✗	✗	✗	✗	✗
Founder + education	✓	✗	✗	✗	✗	✗	✗
Clinical proof (full formula)	✗	✓	✓	✓	●	●	✗
Retail presence	✗	✓	✓	✓	✓	✗	✗
Price / day	SGD 8.50	~0.40	~1.00	~1.00	~4.00	Premium	Mid

4 The premium justification — total cost of ownership (fair, not cheap)

"SGD 8.50 vs Caltrate's 0.40 = 17–28x" is indefensible — the comparator is wrong. A menopausal Asian woman doesn't buy one supplement; she buys a **stack**.

Daily cost — the stack Bonefirm replaces vs Bonefirm vs TCM

THE STACK
(BONE+JOINT+COLLAGEN+MOOD)

SGD 5.80–9.80

Caltrate · Blackmores · Kinohimitsu · Ashwagandha — bought separately

BONEFIRM — ONE PRODUCT

SGD 8.50

Sits inside the stack range — the premium is fair, not a mark-up

TCM HERBAL (CULTURAL DEFAULT)

SGD 7–20

Same root cause (Kidney yin deficiency). Bonefirm is in the same premium band.

The fair-value punchline: a woman covering just the four things Bonefirm addresses already spends **SGD 5.80–9.80/day**. At SGD 8.50 Bonefirm sits *inside* that range — the premium is **fair**. This is a fair-value argument, **not a cheap-price one**.

One product, one root cause — the ingredient–symptom coverage map

WESTERN STRUCTURAL FOUNDATION			TCM BOTANICAL SYSTEM		
INGREDIENT	ADDRESSES	EVIDENCE	INGREDIENT	ADDRESSES	EVIDENCE
NEM®	Joint/connective tissue	Clinical · NCT02751944	Danshen	Bone loss, pain	77–96% osteoporosis
Vitamin C	Collagen synthesis	Established	Chuanxiong	Joint pain, circulation	Pharmacological
			Dendrobium	Bone loss (yin-nourisher)	Prevents bone loss
			Morus · Rosa	Mood / calm	Traditional

✓ Joint pain (#1) — NEM+Danshen+Chuanxiong

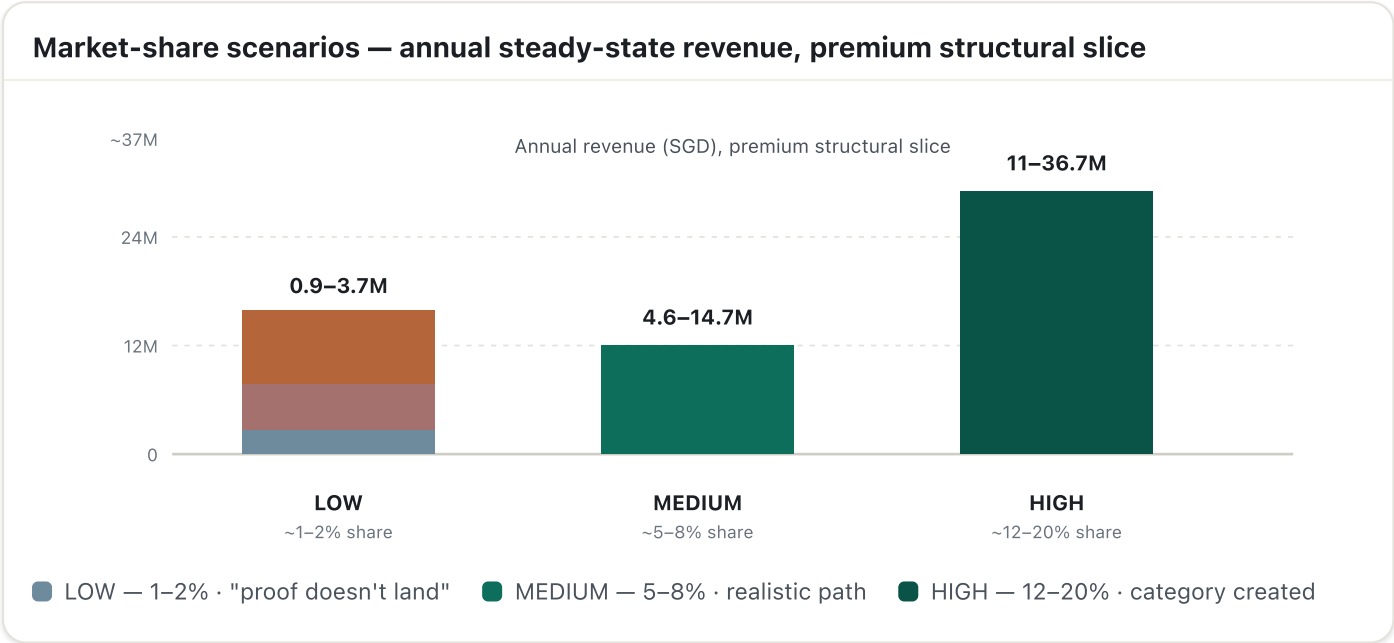
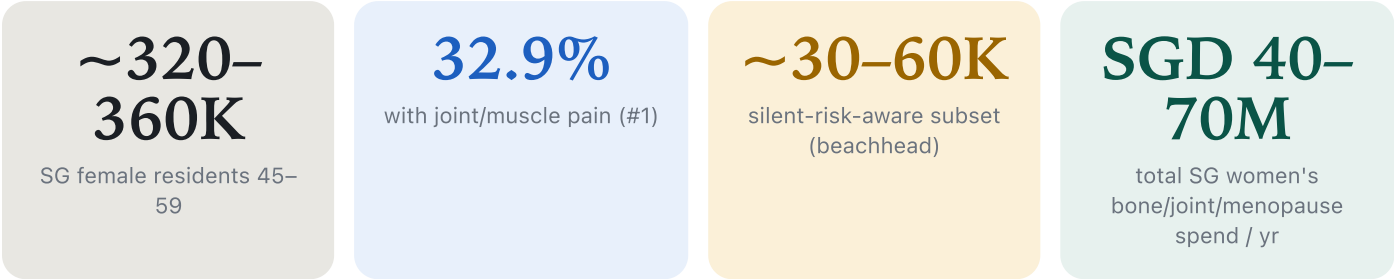
✓ Bone loss (silent) — Danshen+Dendrobium+NEM

🕒 Mood — Rosa+Morus (defensible secondary)

✗ Hot flashes / sleep — NOT claimed

5 Market share — the grab, low/medium/high

The SG women's bone/joint/menopause supplement market is fragmented and under-disclosed. Shares describe **share of purchasing** and overlap — they don't sum to 100%. The premium structural slice Bonefirm targets is small, high-intent, and uncontested.



Scenario table

CASE		KEY ASSUMPTION	SHARE	CUSTOMERS/YR	REVENUE/YR
LOW	risk case	TCO weak; no partnerships; no evidence; competitor enters	~1–2%	~300–1,200	SGD 0.9M–3.7M
MEDIUM	realistic path	TCO holds; 2–3 partnerships; proof page live; education scales	~5–8%	~1,500–4,800	SGD 4.6M–14.7M
HIGH	evidence-backed upside	Medium + formula study + doctor endorsement + category created	~12–20%	~3,600–12,000	SGD 11M–36.7M

Strategic takeaway: the realistic path is **MEDIUM** — own the premium slice (~5–8%) via clinical/physio/TCM partnerships and the NUH/NUS wedge. **HIGH** needs formula-level evidence + a doctor endorsement. **LOW** is the risk if the TCO doesn't hold.

6 Customer acquisition — reach her at the moment of awareness

The beachhead customer is the **silent-risk-aware woman**. Reach her where she is at the moment of awareness — not broadcast to "women 45+". Ranked by intent, reachability, fit with the founder-led model.

RANK	CHANNEL	WHY IT'S THE RIGHT PLACE	INTENT	REACHABILITY
1	DEXA / bone-density clinics	The moment of awareness — just got a scan showing osteopenia, paid SGD 130–150, seeking next steps.	Highest	Med (B2B)
2	Women's-health physios	Treat menopausal joint pain + muscle weakness; already educate on bone density. Endorsement = credibility bridge.	High	Med (B2B)
3	Menopause clinics / gynae	The medical authority. An endorsement clears the credibility gap fastest.	High	Low–Med
4	TCM clinics	The cultural default (37% use herbal). The East–West integration is the natural fit.	High	Med (B2B)
5	RC / community talks	The founder's moat — the education engine feeding everything; where the daughter segment appears.	Medium	High
6	Facebook menopause groups	Peer trust. High-intent but self-promotion restricted — <i>answer, not advertise</i> .	Med–High	Medium
7	Active Ageing Centres / HPB	Mass reach where women 45+ gather; awareness + daughter segment, lower intent.	Low–Med	High

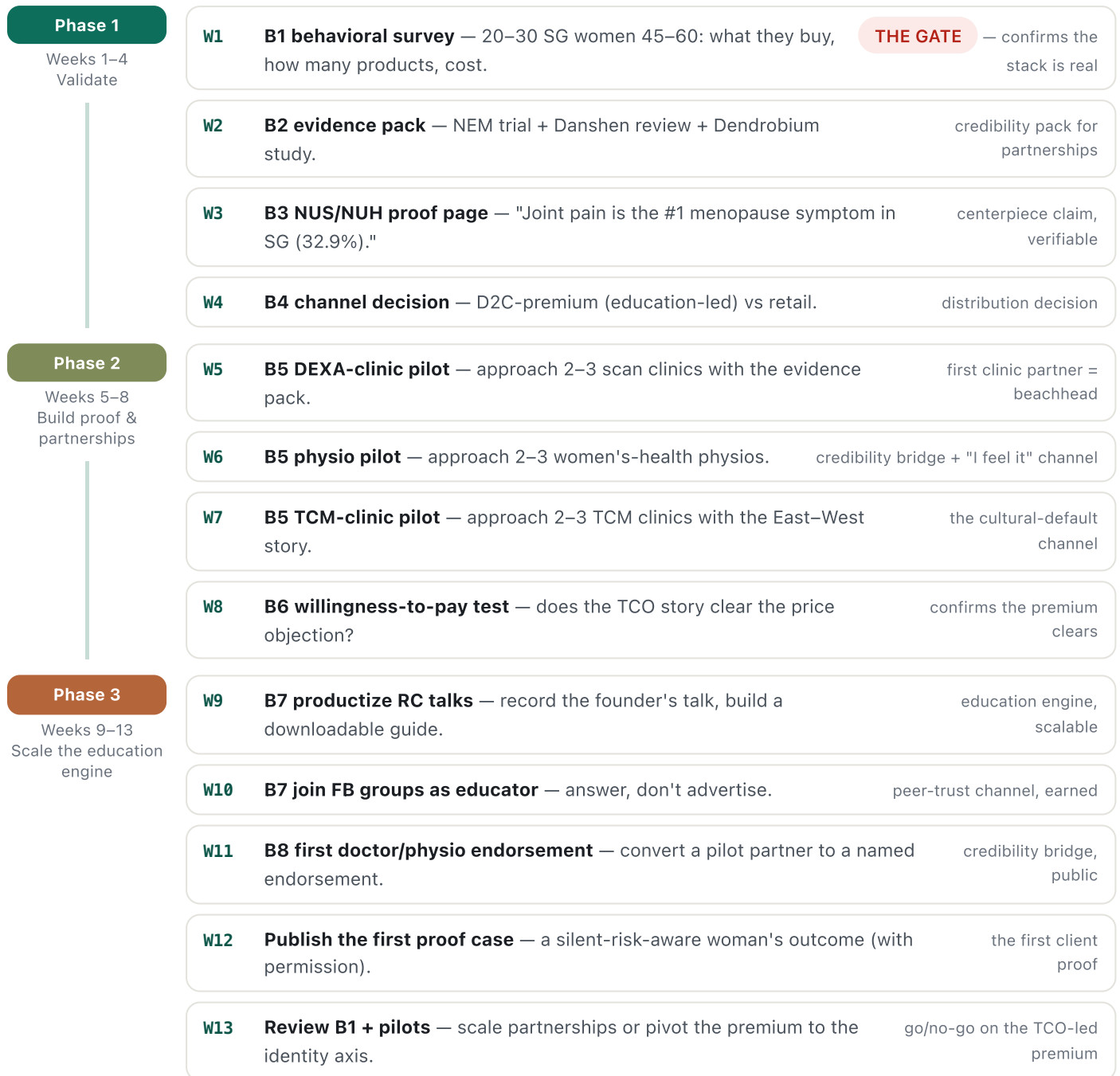
THE RECOMMENDED FOCUS

Lead with the **clinical + TCM partnership channels (1–4)**, powered by the founder's RC talks (5). Partner with DEXA clinics and physios so Bonefirm is handed to the woman **at the moment she's told her bones are at risk — peak intent, with a trusted authority's recommendation**.

Why this beats broadcast: she isn't "browsing supplements" — she's in a clinic, physio, or TCM consult. **Be where the awareness happens, not where she scrolls.** Caution: clinical channels are gatekeepers — B1 and B2 must come first.

7 The 90-day roadmap — validate before you scale (gated on B1)

B1 is the gate — it decides whether the premium holds as a fair price (stack is real) or must lean on the identity axis.



THE 90-DAY GOAL

Validated stack (B1) · evidence pack (B2) · NUS/NUH proof page (B3) · 2–3 clinical/TCM/physio partnerships (B5) · confirmed willingness-to-pay (B6) · first proof case. **The foundation for a defensible premium and a scalable education engine — without spending on broadcast marketing before the proof exists.**

8 Website change recommendations — additions, not a redesign

The site is ~80% aligned with the structural positioning (verified live 2026-08-20).
Changes add evidence, the Asia-first inversion, the premium story, and the mood claim — HSA-safe, in buyer language.

PRIORITY	CHANGE	WHY
P1	Add NUH/NUS proof block + lead homepage with the inversion + three evidence pillars	The wedge — the strongest claim, currently missing
P2	Replace "structural support" with "bones and joints, without hormones"; promote "Non-hormonal by design" to hero	The buyer's words, not analyst jargon
P3	Add "Why the price" section + comparison table (stack 5.80–9.80 vs 8.50) — <i>fair, not cheap</i> ; anchor to the TCM premium band	The price justification, currently absent
P4	Add mood line + name Rosa/Morus; frame as TCM system logic, NOT hot-flash/sleep	Defensible secondary, strengthens TCO
P5	Add "For healthcare professionals" page + education card + partner path	The beachhead channel
P6	Founder story to homepage, first proof case, fix "test2" link, promote Chinese PDF	Credibility + consistency
What NOT to change: no hot-flash or sleep claims · no subscription model (dropped) · no "cheaper than Caltrate" — the price message is fair value, never cheap .		

§ Sources & confidence

VERIFIED CLINICAL (HIGH CONFIDENCE)	ESTIMATES (DIRECTIONAL)
- Logan S et al. <i>Maturitas</i> 178:107853, 2023 · PMID 37806008 — joint pain #1 (32.9%), hot flushes 5th (18.6%) - Logan S et al. <i>Climacteric</i> 2023 · PMID 38099561 — 74.9% any arthralgia - APMF Consensus 2024 · <i>J Menopausal Med</i> 31(1):3–11 — Asia-first inversion - NEM trial NCT02751944 · Danshen review PMID 39421672 · Dendrobium study 2018	- Market size & share — Low / Medium confidence, derived - Stack pricing SGD 5.80–9.80/day — validated SG retail (Straits Times, Kinohimitsu, Watsons/Guardian) - TCM herbal SGD 7–20/day — Eu Yan Sang, Yi TCM, Thomson, Ma Kuang - DEXA ~SGD 130–150, MediSave-claimable — healthscreening.sg, Regis Medical

The one-sentence strategic summary

The NUH/NUS data is the wedge; the silent-risk-aware woman is the beachhead; the education engine is the moat; the fair-value + cultural-anchor TCO is the premium justification; and the 90-day roadmap moves from LOW toward MEDIUM market share, with HIGH as the evidence-backed upside.